

Myomatlas

Decision support for abnormal uterine bleeding

Case	UMD_221129_003
Modality	MRI T2WI sagittal
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PATIENT CONTEXT

Age	58
Menopausal status	Postmenopausal
Bleeding severity	Heavy bleeding
Fertility desire	Fertility not desired

SUMMARY

MRI T2WI sagittal shows six myomas (L in PALM-COEIN). Two transmural lesions (id 1, 27.1 ml, and id 2) reach the endometrium and carry the highest bleeding relevance, provisionally classified as hybrid type 2-5 pending confirmation of the fine 50 percent thresholds. The remaining lesions are subserosal or intramural with low bleeding relevance (ids 3, 4, 5, 6). Critically, this 58-year-old postmenopausal patient with heavy bleeding meets malignancy-exclusion criteria, so endometrial sampling and specialist review are warranted; imaging alone cannot exclude polyp, adenomyosis, or malignancy. Fertility is not desired, so definitive options are appropriate to consider alongside medical and uterus-preserving measures. Final decisions rest with the clinician.

MALIGNANCY ASSESSMENT

Flagged for exclusion

The patient is 58 and postmenopausal with heavy bleeding. Postmenopausal bleeding and age 45 or older with abnormal uterine bleeding both trigger the malignancy_exclusion_rule. Endometrial hyperplasia/malignancy and uterine sarcoma cannot be excluded by imaging; recommend endometrial sampling and specialist referral. This tool does not diagnose or exclude malignancy; a clinician decides.

FINDINGS (6)

Myoma 1, FIGO type 2-5

Hybrid (transmural), impacts endometrium and serosa | Group: Transmural | Confidence: lower (provisional) | Relevance: high

Volume 27.14 mL | Max diameter 51.3 mm | Intramural 53 percent | Contact: cavity, serosa

Transmural: contacts_cavity true (distance_to_cavity_mm 0.0) and contacts_serosa true (distance_to_serosa_mm 0.0), so it reaches both surfaces. Submucosal digit judged as 2 because intramural_pct 53.0 is at/above the 50 percent line and cavity_surface_pct is 0.0 (contact/abutment rather than broad cavity protrusion). Serosal digit judged as 5 because intramural_pct 53.0 is at/above 50 percent with serosa_surface_pct 46.9 (moderate). Both digits sit on the fine 50 percent proxy boundary, hence provisional.

Management options

- Hysterectomy if childbearing is complete and symptoms are refractory
- Tranexamic acid
- Hormonal therapy for bleeding control
- GnRH analogue or antagonist as a bridge
- Combined or staged myomectomy addressing both components

Myoma 2, FIGO type 2-5

Hybrid (transmural), impacts endometrium and serosa | Group: Transmural | Confidence: lower (provisional) | Relevance: high
Volume 2.59 mL | Max diameter 23.2 mm | Intramural 70 percent | Contact: cavity, serosa

Transmural: contacts_cavity true (distance_to_cavity_mm 0.0) and contacts_serosa true (distance_to_serosa_mm 0.0). Submucosal digit 2 because intramural_pct 70.3 is well above 50 percent with only a small cavity-facing surface (cavity_surface_pct 1.2), consistent with contact/minor protrusion. Serosal digit 5 because intramural_pct 70.3 is ≥ 50 percent with serosa_surface_pct 28.5 (moderate). Two-digit designation depends on fine proxy thresholds, hence provisional.

Management options

- Hysterectomy if childbearing is complete and symptoms are refractory
- Tranexamic acid
- Hormonal therapy for bleeding control
- GnRH analogue or antagonist as a bridge
- Combined or staged myomectomy addressing both components

Myoma 3, FIGO type 7

Subserosal, pedunculated; type 6 (subserosal < 50 percent intramural) cannot be excluded from masks | Group: Subserosal | Confidence: lower (provisional) | Relevance: low
Volume 0.77 mL | Max diameter 12.9 mm | Intramural 0 percent | Contact: serosa

contacts_serosa true (distance_to_serosa_mm 0.0), contacts_cavity false (distance_to_cavity_mm 16.2). intramural_pct 0.0 with serosa_surface_pct 100.0 indicates an almost entirely exophytic lesion suggesting type 7, but pedunculation (a true stalk) cannot be confirmed from voxel masks, so type 6 is not excluded.

Management options

- Observation when asymptomatic
- Laparoscopic myomectomy (stalk resection)
- Hysterectomy if childbearing is complete and symptoms are refractory

Myoma 4, FIGO type 5

Subserosal, 50 percent or more intramural | Group: Subserosal | Confidence: moderate | Relevance: low
Volume 0.57 mL | Max diameter 13.2 mm | Intramural 100 percent | Contact: serosa

contacts_serosa true (distance_to_serosa_mm 0.0), contacts_cavity false (distance_to_cavity_mm 8.0). intramural_pct 99.8 is far above 50 percent with only serosa_surface_pct 0.2 exophytic, clearly placing it as type 5 (mostly buried, just reaching the serosa). Well away from the 50 percent boundary so not provisional.

Management options

- Observation when asymptomatic
- Symptomatic management for pressure symptoms
- Myomectomy (laparoscopic or abdominal) for bulk symptoms
- Uterine artery embolization when fertility is not a priority
- Hysterectomy if childbearing is complete and symptoms are refractory

Myoma 5, FIGO type 5

Subserosal, 50 percent or more intramural | Group: Subserosal | Confidence: moderate | Relevance: low
Volume 0.34 mL | Max diameter 9.4 mm | Intramural 87 percent | Contact: serosa

contacts_serosa true (distance_to_serosa_mm 0.0), contacts_cavity false (distance_to_cavity_mm 20.7). intramural_pct 86.7 is well above 50 percent with serosa_surface_pct 13.3, so it reaches the serosa while remaining mostly intramural (type 5). Comfortably clear of the 50 percent line so not provisional.

Management options

- Observation when asymptomatic
- Symptomatic management for pressure symptoms
- Myomectomy (laparoscopic or abdominal) for bulk symptoms
- Uterine artery embolization when fertility is not a priority
- Hysterectomy if childbearing is complete and symptoms are refractory

Myoma 6, FIGO type 4

Intramural, entirely within the myometrium | Group: Intramural | Confidence: higher | Relevance: low
Volume 0.15 mL | Max diameter 7.1 mm | Intramural 100 percent | Contact: none

contacts_cavity false (distance_to_cavity_mm 15.5) and contacts_serosa false (distance_to_serosa_mm 7.2), with intramural_pct 100.0. No contact with either surface is a direct geometric signal for type 4.

Management options

- Observation when asymptomatic
- Tranexamic acid
- Hormonal therapy for bleeding control
- Myomectomy for bulk or bleeding symptoms
- Uterine artery embolization when fertility is not a priority
- Hysterectomy if childbearing is complete and symptoms are refractory

PATIENT EXPLANATION

Your MRI scan shows six fibroids (non-cancerous growths in the muscle of the womb). Two of them (the larger one about the size of a small plum, and one smaller one) reach all the way from the outer surface of the womb through to the inner lining. Fibroids that touch the inner lining are the ones most likely to cause heavy bleeding, so these two are the most likely to explain your symptoms. The other four are smaller and sit within or on the outer surface of the womb wall, and these usually cause little or no bleeding.

Because you are past menopause and are having heavy bleeding, an important safety step is needed: bleeding after menopause always needs to be checked to make sure nothing more serious is going on. A scan alone cannot rule this out. Your doctor will likely recommend taking a small sample of the womb lining and referring you to a specialist. This is a routine precaution, not a diagnosis.

Since you have told us you do not wish to become pregnant, there are several options your care team may discuss. These range from medicines to control the bleeding (such as tranexamic acid, hormone treatments, or a short course of a hormone-blocking medicine), to procedures that treat the fibroids while keeping the womb, up to removing the womb entirely (hysterectomy), which is a permanent solution when symptoms are severe and other treatments have not helped.

Some things cannot be seen on this type of scan alone, such as small lining polyps or changes in the muscle of the womb, so further tests may be suggested. This report is a support tool only, and your doctor will make the final decisions with you.

IMPORTANT

This report is decision support, not a diagnosis. A clinician makes the final diagnosis and treatment decision.